

The term "health disparities" describes a systematic incidence of poorer health outcomes for groups that have traditionally experienced social disadvantage (eg, racial and ethnic minorities, lower-income individuals, women) as compared to more advantaged groups (eg, whites, higher-income individuals, men). These health disparities are driven by unequal access to health care services and health insurance due to prejudice and discrimination on the part of health care providers and the overall system, and extensive research has focused on these issues.

Recent research suggests that another factor might explain health disparities: stereotype threat on the part of the patient. Stereotype threat can be evoked by verbal or nonverbal cues from health care professionals, such as a physician's use of confusing medical jargon (the standardized language medical personnel use to communicate among themselves) or a nurse's body language while discussing the risks of obesity and alcohol use. Once triggered, stereotype threat may result in behaviors that compromise one's own health. For example, a patient may withhold information or feel compelled to describe health-related behaviors in a positive light. Similarly, if the interaction arouses anxiety, a patient may be less likely to remember the physician's instructions or might dismiss a physician's advice as biased.

Stereotype threat can be reduced through training programs that improve employees' communication skills. Some hospitals in large urban areas have implemented the use of cultural liaisons, individuals who are able to bridge the gap between a minority patient's cultural beliefs and Western medicine. Cultural liaisons are often present during the nonsensitive portions of the doctor's appointment, and studies suggest they may help to reduce stereotype threat in patients.

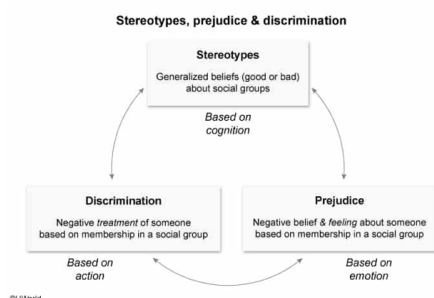
Which of the following scenarios is most consistent with the description of prejudice in the first paragraph? A physician:

- ☐ A. asks minority patients about their drinking and smoking habits. [1%]
- ☐ B. follows up with minority patients more frequently due to concern that they will be noncompliant. [2%]
- ☒ C. dislikes treating minority patients due to the assumption that they are poorly educated about their health. [67%]
- ☐ D. refuses to write narcotic prescriptions for minority patients due to suspicion that they will misuse the drugs. [29%]

Correct

67% answered correctly

Explanation:



A **stereotype** is an overly simplified, generalized characterization of a particular group of people. Stereotypes can be positive (eg, Asians are good at math) or negative (eg, women are bad at math).

Whereas stereotypes are primarily cognitive (belief-based), **prejudice** involves **negative beliefs** and **feelings**

toward members of a group (eg, a dislike of working with women based on the belief that they are bad at math).

Discrimination involves **negative treatment** of individuals or groups based on prejudice or stereotypes. A hiring manager who believes that women are bad at math (stereotype), who dislikes when females apply for accounting positions (prejudice), and who therefore does not hire women is demonstrating discrimination.

According to the first paragraph, health disparities are driven by unequal access to health care services and health insurance due (in part) to *prejudice*. A scenario in which a physician dislikes treating certain patients due to an assumption about them (ie, prejudice) is most consistent with prejudice that might result in health disparities.

(Choice A) Asking minority patients about their drinking and smoking habits does not necessarily reflect prejudice because these are reasonable health-related questions. If the physician asked *only* minority patients about drinking and smoking habits, this would be an example of discrimination.

(Choice B) Following up with minority patients due to fear of noncompliance is not an example of prejudice because it involves the physician's behavior, not merely a belief. In addition, this behavior is likely to *improve* (not negatively impact) health outcomes for minority patients.

(Choice D) Refusal to write narcotic prescriptions for minority patients based on the belief that minorities are more likely to misuse drugs is an example of discrimination, not prejudice.

Educational objective:

Stereotypes are overgeneralized beliefs about groups and can be positive or negative. Prejudice involves negative beliefs and feelings about members of a group. Discrimination involves actions based on stereotypes and prejudice that negatively impact a group.

Time spent:0 sec

Subject:
Behavioral Sciences

QID:401053

System:
Identity and Social Interaction

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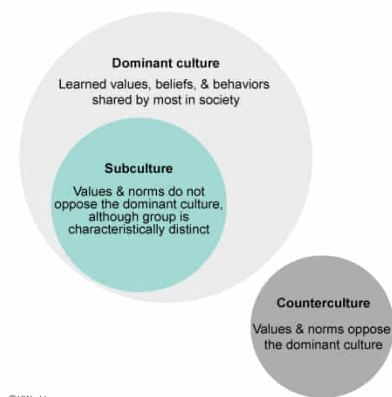
Stereotype threat can be reduced through training programs that improve employees' communication skills. Some hospitals in large urban areas have implemented the use of cultural liaisons, individuals who are able to bridge the gap between a minority patient's cultural beliefs and Western medicine. Cultural liaisons are often present during the nonsensitive portions of the doctor's appointment, and studies suggest they may help to reduce stereotype threat in patients.

Based on the description of medical jargon in the second paragraph, medical personnel can best be described as which of the following?

- ☐ A. A counterculture [6%]
- ☐ B. An aggregate [12%]
- ☐ C. A primary group [14%]
- ☒ D. A subculture [66%]

Correct
66% answered correctly

Explanation:



A **subculture** is a group of individuals who are characteristically **distinctive** from the dominant culture in some way, but whose values and norms still align with the dominant culture. The **dominant culture** includes the established set of norms, values, rituals, and beliefs that define a society.

The use of medical jargon, the standardized language used to communicate within a group, suggests that medical personnel can best be described as a subculture. Members of a subculture tend to identify as a group, have their own jargon and mode of dress (eg, scrubs, white coats, stethoscopes), and engage in characteristically distinctive behaviors (eg, treating patients, being on call).

(Choice A) A counterculture is a group of individuals whose norms and values *oppose* those of the dominant culture (eg, the hippie counterculture was opposed to the Vietnam War, which was largely supported by the dominant culture at the time). The use of medical jargon does not suggest that medical personnel oppose the dominant culture.

(Choice B) An aggregate is a collection of individuals who share a common location but do not identify as a group (eg, all the people at a given cafe at 6:00 AM). Medical personnel would be considered a group, not an aggregate.

(Choice C) A primary group is composed of members who have frequent contact and are emotionally connected (eg, close friends, family members). Secondary groups are composed of members who interact for a common goal (eg, colleagues). Medical personnel who work together would be considered a secondary (not primary) group.

Educational objective:

The dominant culture establishes the norms, values, and rituals of a society. A subculture is distinct from the dominant culture in some way but is still aligned with the norms and values of the dominant culture. A counterculture opposes the norms and values of the dominant culture.

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Subject:
Behavioral Sciences

QID:401054
System:
Identity and Social Interaction

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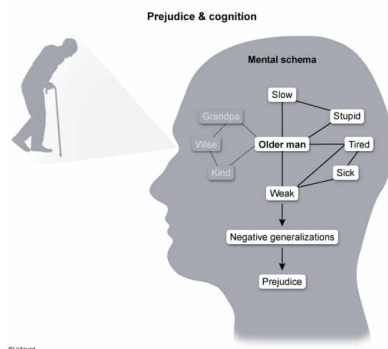
Which of the following would most directly test the hypothesis that cognition plays a role in physicians' prejudice?

- ✓ ☒ A. Using an implicit association test to determine if physicians have negative biases toward certain patients [79%]
- ☐ B. Recording physicians' physiological responses (pupil dilation, skin conductance) while interacting with different patients [9%]
- ☐ C. Surveying patients immediately after a doctor's appointment on the quality of their interaction with their physician [4%]
- ☐ D. Observing and recording the number of negative interactions between physicians and their patients [6%]

Correct

78% answered correctly

Explanation:



Cognition plays a role in prejudice because the brain is wired to quickly categorize things (including people) using **schemas**, which are **mental frameworks** that organize old information and allow quick processing of new information. Our brains tend to unconsciously and automatically categorize people based on their most obvious social characteristics: age, race/ethnicity, and gender.

The **implicit association test (IAT)** is a psychometric technique designed to **measure unconscious attitudes**. The test requires participants to match words or images to one of two opposite categories as quickly as possible. Faster response times imply a stronger association between the word and image because the participant does not have to deliberate about a decision for very long.

The IAT would most directly test the role of cognition in prejudice because it provides a measure of unconscious cognitive associations, which may reflect negative feelings and beliefs about certain social groups (prejudice).

(Choice B) Physiological responses like pupil dilation and skin conductance are good biological markers of *emotion*, not cognition. Therefore, these physiological responses would provide more information about physicians' emotional responses to a patient rather than their cognitive responses.

(Choice C) Surveying patients might help clarify *patients' prejudice* toward doctors and the health care system, but it would not be the best way to directly test physicians' prejudices, which may not be obvious or visible to the patients.

(Choice D) Negative interactions between physicians and patients would provide information about discrimination or stereotype threat, but they would not be the best way to directly test physicians' prejudices, which may not be obvious or visible.

Educational objective:

The role of cognition in prejudice results from the quick, unconscious categorization of people using schemas based on age, race/ethnicity, and gender. The Implicit Association Test (IAT) is a psychometric technique designed to measure unconscious attitudes, including prejudice (negative feelings and beliefs).

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Subject:
Behavioral Sciences

QID:401055

System:
Identity and Social Interaction

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Based on the first paragraph, which of the following best defines institutional discrimination?

- ☐ A. Differential treatment of certain group members that is intentional [4%]
- ☐ B. Behavior that is based on stereotypes about group members [2%]
- ☐ C. Behavior that is affected by stereotype threat
- ✓ ☒ D. Differential treatment of certain group members occurring at the system level [92%]

Correct

92% answered correctly

Explanation:

Institutional discrimination refers to differential treatment of specific social groups (eg, racial/ethnic minorities) at the **organizational** or **system level**, resulting in negative consequences for those affected. Institutional discrimination includes actions and policies that are often subtle and unintentional but nevertheless **cause harm**.

Institutional discrimination can take place within any institution, including schools, hospitals, and corporations. For example, if employees are promoted only if they do not take extended time off from work, thereby preventing women who take maternity leave from being promoted, this would be an example of institutional discrimination.

Institutional discrimination is best defined as differential treatment of certain group members occurring at the system level.

(Choice A) Institutional discrimination includes systemic actions and policies that are intentional or

unintentional. Even unintentional actions and policies can cause harm.

(Choice B) Stereotypes can be positive or negative, but discrimination is always negative/harmful. Furthermore, *institutional* discrimination occurs on the systemic level, whereas individual discrimination involves behavior that is based on negative stereotypes about group members.

(Choice C) Interactions between individuals that may result in stereotype threat does not best define institutional discrimination, which occurs at the systemic level.

Educational objective:

Institutional discrimination occurs when members of a certain group are negatively impacted because of systematic differential treatment. Institutional discrimination can be intentional or unintentional.

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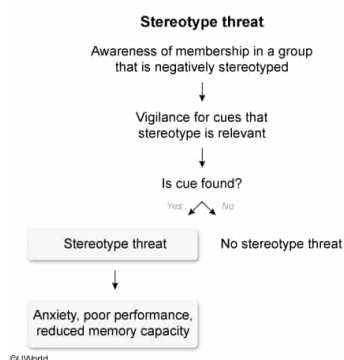
Which of the following concepts is LEAST relevant to the processes described in the second paragraph?

- ☐ A. Self-concept [11%]
- ✓ ☒ B. Cultural relativism [45%]
- ☐ C. Autonomic arousal [28%]
- ☐ D. Self-fulfilling prophecy [15%]

Correct

44% answered correctly

Explanation:



Stereotype threat refers to the **anxiety** experienced by an individual who feels judged based on a negative stereotype about a group to which he or she belongs. For stereotype threat to occur, the individual must be made **aware** of his or her **membership** in a group that is **negatively stereotyped**. This awareness arouses vigilance, which often results in anxiety that negatively impacts performance.

Stereotype threat can result from an interaction between a patient and physician if, for example, the patient is made aware of belonging to a group that is stereotypically associated with alcoholism. The patient might underreport consumption of alcohol for fear of validating the stereotype about the group.

Cultural relativism, in which there are no "right" or "wrong" cultural practices, is most inclusive of cultural differences. Stereotype threat is less likely to occur in a health care situation that is culturally relativistic.

(Choice A) Self-concept refers to beliefs about oneself, including beliefs about race/ethnicity, gender, ability, talent, and so on. Awareness of one's membership in a group (part of one's self-concept) that is negatively stereotyped is key to triggering stereotype threat.

(Choice C) Stereotype threat evokes feelings of anxiety, which are mediated by the sympathetic branch of the autonomic nervous system.

(Choice D) Self-fulfilling prophecy occurs when a belief about something (that may or may not be true) influences the behavior of the person who believes it, resulting in an outcome that may validate the belief as true. The anxiety produced by stereotype threat can cause poor performance (ie, self-fulfilling prophecy).

Educational objective:

Stereotype threat occurs when an individual is made aware of a negative stereotype regarding a group with which that individual identifies, and this awareness causes the individual's performance to suffer.

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Subject:
Behavioral Sciences

QID:401057

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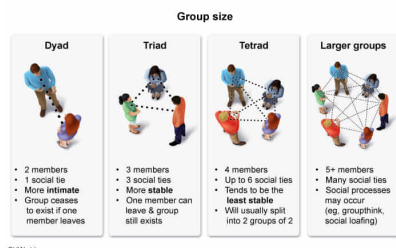
Besides reducing stereotype threat, the presence of a cultural liaison during a doctor's appointment increases:

- ☐ A. the number of group members, making groupthink more likely. [5%]
- ☐ B. the number of group members, making group polarization more likely. [4%]
- ✓ ☒ C. the number of social ties, making the group more stable. [71%]
- ☐ D. the number of social ties, making the group more intimate. [18%]

Correct

71% answered correctly

Explanation:



Social groups are composed of individuals who interact and identify with each other. As group size increases, the number of potential **social ties** (relationships between individual group members) also increases. Social groups can be large (eg, whole societies) or small.

A **dyad** (relationship between two people) has only one social tie, making dyadic relationships the most intimate (eg, romantic couples, business partners); however, dyads are also less stable than larger groups because if either person leaves, the group ceases to exist.

A **triad** (relationship among three people) can have three potential social ties, making triadic relationships more stable but less intimate than dyads. The presence of a cultural liaison increases the number of social ties, resulting in a more stable group.

(Choice A) Groupthink, irrational decision making by groups that value reaching a consensus over critical evaluation of information, is more likely to occur when all group members are similar. A group composed of a physician and patient (who have dissimilar backgrounds) and a cultural liaison is unlikely to be prone to groupthink.

(Choice B) Group polarization occurs when group members adopt a more extreme attitude or course of action after group discussion. For group polarization to occur, group members must share similar opinions prior to group discussion, which is not necessarily true of a group composed of a patient, physician, and cultural liaison.

(Choice D) The added group member would make the group less, not more, intimate.

Educational objective:

Individuals who interact and identify with each other form social groups. The relationships between individual group members are social ties. Dyads (groups of two) have one social tie and are more intimate but less stable than triads. Triads (groups of three) have three potential social ties and are less intimate but more stable than dyads.

Time spent:0 sec
Subject:
Behavioral Sciences

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System:
Identity and Social Interaction